

Alcohol Withdrawal

Inpatient management for IM residents — predict, treat, escalate, support

Step 1 — Predict & place on the timeline

PAWSS on admission; know the clock

Onset	Syndrome
6–24 h	Minor: tremor, anxiety, ↑HR/BP, diaphoresis
12–48 h	Withdrawal seizures (generalized tonic-clonic)
12–48 h	Alcoholic hallucinosis (CLEAR sensorium)
48–96 h	Delirium tremens (confusion + autonomic storm)

PEARL Hallucinosis = clear sensorium + normal vitals. DTs = altered sensorium + autonomic instability.

Step 2 — Benzodiazepines first-line

Symptom-triggered, fixed, or front-loaded

- Benzodiazepines reduce seizures and DTs — first-line (ASAM)
- Symptom-triggered (CIWA-Ar): less total drug, shorter treatment
- Front-load a long-acting benzo for high-risk/severe presentations
- Long-acting (diazepam/chlordiazepoxide) self-tapers
 - Use lorazepam in significant liver disease (no active metabolites)
- CIWA-Ar is INVALID if the patient can't be assessed

PEARL A high CIWA in a non-communicative patient is noise — switch to fixed/front-loaded/phenobarbital.

Step 3 — Severe / refractory: phenobarbital

For benzodiazepine-resistant or severe withdrawal

Escalation	Note
Phenobarbital	Severe/refractory; long half-life, self-tapering; GABA + glutamate
Dexmedetomidine / propofol	ICU adjuncts — do NOT prevent seizures alone
ICU transfer	DTs, airway risk, escalating requirements

PEARL Adjuncts treat autonomics/agitation but not seizures — keep a GABAergic agent on board.

Support — vitamins & the basics

- THIAMINE before glucose — dextrose first can precipitate Wernicke's
- Replete magnesium, potassium, phosphate (Mg supports seizure threshold)
- Screen for infection, GI bleed, head injury, pancreatitis, liver disease
- Treat the alcohol use disorder: naltrexone/acamprosate + linkage

PEARL Thiamine is cheap and safe — give it generously and early in malnourished drinkers.

What to avoid

DO

- Risk-stratify with PAWSS
- Thiamine before glucose; replete Mg
- Phenobarbital for benzo-resistant/severe
- Keep a GABAergic agent on board

AVOID

- Dosing off CIWA in an unassessable patient
- Clonidine/dexmed/antipsychotic alone
- Glucose before thiamine
- Undertreating — DTs carry real mortality

PEARL The two classic errors: glucose-before-thiamine and titrating off an invalid CIWA.

Take-home points

- PAWSS to predict; seizures 12–48 h, DTs 48–96 h
- Benzodiazepines first-line; symptom-triggered when CIWA is valid
- Phenobarbital for severe/benzo-resistant withdrawal
- Adjuncts don't prevent seizures — keep GABAergic coverage
- Thiamine before glucose; replete Mg; link to AUD treatment